NXT LVL | SIMPLIFIED EDITION

Peptide research handbook

A concise guide to the **31 individual catalog listings and six research packages** checked on November 3, 2026. Different vial amounts are grouped under the same material. [1, 2]

Includes what each material is studied for, the strength of the evidence, reconstitution basics, mixing ratios and combination cautions.

Read this first

Research findings do not establish safe human use. An approved medicine and a research vial with the same ingredient name are not interchangeable. Use the exact product instructions for preparation and storage; this guide does not supply treatment doses.

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THE BASICS

Reconstitution and storage

Reconstitution means adding the specified liquid to a dry preparation. It does not confirm identity, remove contamination or make a research powder sterile.

A simple laboratory workflow

1 Check the vial. Confirm the material, amount, lot, expiry and container condition. For a blend, obtain the amount of every ingredient.

2 Check the instructions. Use the solvent, final concentration and mixing method specified for that exact formulation. Bacteriostatic water is not suitable for every material.

3 Prepare and measure. Use the required clean laboratory setup and accurate measuring equipment. Keep the volume added distinct from the final solution volume.

4 Mix as directed. Follow the specified method. Do not improvise heating, vigorous shaking, acids or extra solvents when material does not dissolve.

5 Inspect. Compare with the expected appearance. Hold any preparation with unexplained particles, cloudiness or color change. A clear solution can still be unsuitable.

6 Label and store. Record material, lot, solvent, concentration, preparation date, storage conditions and the validated use-by time.

This is a laboratory checklist, not a validated preparation method for the catalog. For a prescribed medicine, use its label or pharmacist instructions. [5, 6, 7]

Storage rules worth keeping

No universal shelf life. Dry-vial expiry does not tell you how long a mixed solution lasts. Refrigeration or freezing must be supported by the actual product instructions.

Preservative is not sterilization. Bacteriostatic water does not make contaminated powder safe or remove endotoxin. The familiar 28-day multidose-vial rule does not validate every peptide solution. [7, 39]

Stop when information is missing. Resolve an unknown concentration, damaged vial, storage excursion or incomplete blend formula before continuing.

QUICK MATH

Mixing ratios and calculations

10 mg is an amount of material. 10 mL is a volume. A container marked 10 mL does not tell you how many milligrams it contains.

Concentration = total mg ÷ final mL
Material in a measured portion = mg/mL × portion in mL
1 mg = 1000 micrograms (mcg).

Your 10 mg example

If a vial contains **10 mg** and the **final volume is 1 mL**, every **0.1 mL contains 1 mg**. Here is how that changes with other final volumes:

Total material	Final volume	Concentration	Amount in 0.1 mL
10 mg	1 mL	10 mg/mL	1 mg / 1000 mcg
10 mg	2 mL	5 mg/mL	0.5 mg / 500 mcg
10 mg	3 mL	3.333 mg/mL	About 0.333 mg / 333 mcg
10 mg	4 mL	2.5 mg/mL	0.25 mg / 250 mcg
10 mg	5 mL	2 mg/mL	0.2 mg / 200 mcg
10 mg	10 mL	1 mg/mL	0.1 mg / 100 mcg

These are arithmetic examples, not recommended mixing volumes or doses. They assume accurate content and complete dissolution. The solvent must be suitable and the vial must accommodate the volume. Volume added can differ from final volume.

Two more examples

20 mg in 2 mL = 10 mg/mL; 0.1 mL contains 1 mg.
1 mg in 2 mL = 0.5 mg/mL; 0.1 mL contains 0.05 mg, or 50 mcg.

Syringe markings and IU

On a syringe specifically marked **U-100**, 100 scale units = 1 mL and 10 scale units = 0.1 mL. Those markings are not milligrams or HCG IU. Read the actual device scale.

Blends: a total weight does not reveal each ingredient’s concentration. You need the individual amounts first.

CATALOG 1 OF 7

Tissue research and blends

BPC-157

CATALOG AMOUNT 10 mg

Studied benefit. Tendon, muscle and gastrointestinal repair research, mainly from laboratory and animal work.

Key point. Human evidence remains limited. It is not established as a treatment for torn tendons, injuries or inflammatory bowel disease. [10]

TB-500

CATALOG AMOUNT 10 mg

Studied benefit. Tissue and wound research related to thymosin biology.

Key point. Verify the exact molecule. The evaluated TB-500 fragment is not the same as full-length thymosin beta-4, and clinical wound-healing benefit is unestablished. [11]

TB-500 + BPC-157 blend

CATALOG AMOUNT 10 mg total

Studied benefit. Combines two tissue-research materials in one vial.

Key point. The website does not specify each ingredient’s mass. Do not assume 10 mg of each or an equal split. Combination benefit is unestablished. [10, 11, 41]

GLOW

CATALOG AMOUNT 70 mg total

Studied benefit. A named blend whose intended effects depend on its ingredients.

Key point. The website does not publish the full formula. Benefits, ingredient concentrations and compatibility cannot be assigned from the name alone. [40]

CATALOG 2 OF 7

Skin and metabolic research

GHK-Cu

CATALOG AMOUNT 50 mg and 100 mg

Studied benefit. Copper peptide studied for collagen production and tissue remodeling in cells and animals.

Key point. These findings do not prove systemic anti-aging or hair-regrowth benefits. Topical evidence does not establish injectable safety; the two sizes are the same named material. [3, 12, 13]

KPV

CATALOG AMOUNT 10 mg

Studied benefit. A three-amino-acid peptide studied for inflammation-related effects in cells and animals.

Key point. Human benefit and safety are not established. It is not a proven gut, skin or general inflammation treatment. [14]

MOTS-c

CATALOG AMOUNT 10 mg

Studied benefit. A mitochondria-derived peptide studied for metabolic regulation and insulin sensitivity in preclinical models.

Key point. Human administration data remain inadequate. It is not established for energy, weight loss, diabetes or longevity. [24, 25]

AOD-9604

CATALOG AMOUNT 10 mg

Studied benefit. A modified growth-hormone fragment investigated for fat metabolism.

Key point. Human research did not establish the intended weight-loss benefit. Animal effects do not prove fat loss, spot reduction or effectiveness in a stack. [26]

CATALOG 3 OF 7

Weight and energy research

Semaglutide

CATALOG AMOUNT 10 mg

Studied benefit. Activates GLP-1 receptors. Specific prescription products have established diabetes, weight-management and other labeled benefits.

Key point. A research vial is not the approved medicine. Major concerns include gastrointestinal effects, pancreatitis, gallbladder problems and thyroid-related restrictions. Avoid overlapping GLP-1 regimens. [4, 16]

Tirzepatide

CATALOG AMOUNT 10 mg

Studied benefit. Activates GIP and GLP-1 receptors. Specific prescription products have established diabetes, weight-management or obesity-related sleep-apnea uses.

Key point. Similar gastrointestinal and other serious risks apply. Zepbound labeling advises against combining it with other tirzepatide products or GLP-1 receptor agonists. [4, 17]

Retatrutide

CATALOG AMOUNT 10 mg and 20 mg

Studied benefit. Investigational GLP-1, GIP and glucagon agonist. Clinical trials and 2026 sponsor-reported phase 3 results showed weight and glucose-control effects.

Key point. It remains investigational in the cited review. Trial results do not certify research vials. Gastrointestinal effects and heart-rate increases are concerns. [4, 15, 43]

SLU-PP-332

CATALOG AMOUNT 10 mg

Studied benefit. A nonpeptide small molecule studied for metabolism and endurance effects in cells and mice.

Key point. Human benefit and safety are unestablished. It is not a proven exercise replacement; freeze-drying does not make it suitable for a peptide-water recipe. [38]

CATALOG 4 OF 7

Growth hormone pathways

Tesamorelin

CATALOG AMOUNT 10 mg

Studied benefit. Stimulates GH release. Approved Egrifta formulations reduce excess abdominal fat in adults with HIV-associated lipodystrophy.

Key point. This is a specific indication, not general weight-loss or bodybuilding proof. Risks include higher IGF-1, glucose intolerance and fluid retention. Mixing instructions vary by formulation. [8, 9]

Sermorelin

CATALOG AMOUNT 10 mg

Studied benefit. A GHRH fragment that stimulates the pituitary's release of growth hormone; historical prescription products had growth-related uses.

Key point. That history does not establish adult anti-aging, recovery or research-vial benefits. More GH signaling does not automatically mean better outcomes. [18, 20]

CJC-1295 with DAC

CATALOG AMOUNT 10 mg

Studied benefit. A longer-acting GHRH analogue. Small human trials showed prolonged GH and IGF-1 increases.

Key point. Hormone-marker changes do not prove muscle gain, fat loss or longevity. Clinical evidence is limited; cardiovascular reactions have been reported. [3, 19, 20]

CJC-1295 without DAC

CATALOG AMOUNT 10 mg

Studied benefit. A non-DAC GHRH-related material, often marketed as modified GRF 1-29.

Key point. Confirm identity. Findings for the longer-acting DAC form cannot simply be transferred to this product. Clinical benefits and combination safety remain unestablished. [19, 20]

CATALOG 5 OF 7

Growth, sleep and anxiety

Ipamorelin

CATALOG AMOUNT 10 mg

Studied benefit. Stimulates GH release through the ghrelin-receptor pathway, different from the GHRH pathway.

Key point. Early pharmacology does not prove better recovery, sleep or muscle gain. Route-specific safety information is limited, and pairing it with a GHRH analogue is not validated by mechanism alone. [3, 21]

IGF-1 LR3

CATALOG AMOUNT 1 mg

Studied benefit. A modified IGF-1 analogue with growth-related effects in experimental studies.

Key point. It is not the prescription medicine mecasermin. Human muscle-building benefit is unestablished. Low blood glucose and growth-related effects are important biological concerns, especially with overlapping agents. [22, 23]

DSIP

CATALOG AMOUNT 10 mg

Studied benefit. Delta sleep-inducing peptide, also called emideltide, investigated in sleep and withdrawal research.

Key point. Small older studies did not establish effectiveness for insomnia, narcolepsy or opioid withdrawal. Its name does not guarantee better sleep; combinations with sedatives are not validated. [29]

Selank

CATALOG AMOUNT 11 mg

Studied benefit. Investigated for anxiety-related effects; a small comparative human study reported symptom improvement.

Key point. Evidence is limited and does not establish broad anxiety treatment, cognitive enhancement or injectable research-vial safety. A Selank/Semax stack is unproven. [3, 27]

CATALOG 6 OF 7

Brain, pigmentation and desire

Semax

CATALOG AMOUNT 11 mg

Studied benefit. Investigated in neurologic research, including ischemia, migraine and neuralgia.

Key point. The reviewed evidence did not establish effectiveness. Most human reports involved nasal use; those findings do not validate injections. Safety and possible bleeding concerns remain. [28]

Melanotan-1

CATALOG AMOUNT 10 mg

Studied benefit. Generally refers to afamelanotide. Its regulated implant helps adults with erythropoietic protoporphyria tolerate light exposure.

Key point. A research vial is not the approved implant or an established cosmetic-tanning treatment. Pigmentation changes and skin monitoring matter; sun protection is still needed. [30]

Melanotan-2

CATALOG AMOUNT 10 mg

Studied benefit. Early small human research observed pigmentation and sexual effects.

Key point. Reliable long-term benefit and safety are unestablished. Nausea, fatigue and prolonged erections are concerns. Overlap with PT-141 does not establish a useful combination. [3, 31]

PT-141

CATALOG AMOUNT 10 mg

Studied benefit. Bremelanotide affects melanocortin pathways. Vyleesi has a specific low-sexual-desire indication in certain premenopausal women.

Key point. This does not establish general libido or male performance benefits. Risks include nausea, pigmentation and blood-pressure changes; cardiovascular restrictions apply to the approved product. [32]

CATALOG 7 OF 7

Hormones and related materials

HCG

CATALOG AMOUNT 5000 IU and 10000 IU

Studied benefit. A reproductive hormone with established prescription uses in selected fertility and hormone-deficiency settings.

Key point. It is not a weight-loss aid or guaranteed fertility-restoration product. Risks include ovarian hyperstimulation, fluid retention and gynecomastia. IU measure activity, not volume. [33]

Glutathione

CATALOG AMOUNT 1.5 g / 1500 mg in a listed 10 mL vial

Studied benefit. An antioxidant tripeptide. Some oral studies reported changes in skin measurements.

Key point. This does not establish injectable “detox” or recovery benefits. Endotoxin-contaminated preparations have caused reactions. Vial capacity is not solution concentration. [1, 36, 37]

NAD+

CATALOG AMOUNT 500 mg

Studied benefit. A coenzyme involved in cellular metabolism; it is not a peptide. Small infusion research measured metabolic changes.

Key point. That does not prove improved energy, anti-aging or longevity. Injectable-product contamination and endotoxin reactions are concerns. [34, 35]

Bacteriostatic Water

CATALOG AMOUNT 10 mL

Studied benefit. A preserved diluent for a product or method that specifically requires it.

Key point. It is not a therapeutic peptide, a universal solvent or a sterilizer. Verify the actual product’s quality, preservative and compatibility. [7, 39]

COMBINATIONS

Stacks and website packages

These six packages contain **separate vials**. Their themes explain why they are discussed together; none is established as a safe or effective human treatment stack by the reviewed sources. [2]

Website package	Rationale and main limitation
BPC-157 10 mg + TB-500 10 mg	Tissue-research theme. Better results from the combination are unestablished. Different from the 10 mg total blend. [10, 11]
CJC-1295 without DAC 10 mg + Ipamorelin 10 mg	Different receptor pathways involved in GH release. Clinical synergy and combination safety are unestablished. [20, 21]
Tesamorelin 10 mg + Ipamorelin 10 mg	Overlapping GH effects. Tesamorelin’s approved-product evidence does not validate this pairing. [9, 21]
GHK-Cu 50 mg + GHK-Cu 100 mg	Same named material in two amounts. Additional quantity, not a complementary stack. [1]
BPC-157 10 mg + KPV 10 mg	Inflammation and tissue-response theme. Clinical combination benefit is unestablished. [10, 14]
Sermorelin 10 mg + Ipamorelin 10 mg	Two routes to GH stimulation. Not a validated addition to the other GH packages. [18, 21]

Three rules to remember

Do not assume overlapping drugs work better. Semaglutide, tirzepatide and retatrutide share GLP-1 activity. Multiple GH-pathway products and PT-141/Melanotan-2 also overlap. [9, 17, 20, 31, 32]

A stack is not a mixing instruction. Same-vial compatibility requires data for the exact ingredients, solvent, concentrations and storage conditions.

Unknown formula means unknown overlap. Confirm GLOW’s ingredients and each blend component’s amount before interpreting any combination. [40, 41]

REFERENCE LINKS

Sources

Selected primary studies, official labels and agency reviews checked November 3, 2026. Numbers match the text. Click a source title to open it. Labels describe their named regulated products, not NXT LVL stock.

[1] NXT LVL Research
Live catalog and home collection

[2] NXT LVL Research
Research package deals

[3] FDA
Bulk substances that may present significant safety risks

[4] FDA
Concerns with unapproved GLP 1 drugs

[5] CDC
Preventing unsafe injection practices

[6] Sigma Aldrich
Peptide handling guide

[7] DailyMed
Bacteriostatic Water for Injection Hospira

[8] DailyMed
EGRIFTA SV tesamorelin

[9] DailyMed
EGRIFTA WR tesamorelin

[10] FDA
BPC 157 briefing document

[11] FDA
TB 500 briefing document

[12] Maquart and colleagues
GHK Cu and collagen synthesis in fibroblasts

[13] Maquart and colleagues
GHK Cu and connective tissue in rat wounds

[14] FDA
KPV briefing document

[15] Jastreboff and colleagues
Retatrutide for obesity phase 2 trial

[16] DailyMed
WEGOVY semaglutide

[17] DailyMed
ZEPBOUND tirzepatide

[18] FDA Federal Register
GEREF sermorelin withdrawal determination

[19] Teichman and colleagues
CJC 1295 stimulation of GH and IGF I

[20] FDA
CJC 1295 briefing document

[21] Raun and colleagues
Ipamorelin as a selective GH secretagogue

[22] Tomas and colleagues
Potency of IGF I analogues in rats

REFERENCE LINKS

Sources continued

Selected primary studies, official labels and agency reviews checked November 3, 2026. Numbers match the text. Click a source title to open it. Labels describe their named regulated products, not NXT LVL stock.

[23] DailyMed
INCRELEX mecasermin

[24] Lee and colleagues
MOTS c metabolic homeostasis

[25] FDA
MOTS c briefing document

[26] FDA
AOD 9604 briefing document

[27] Medvedev and colleagues
Selank and phenazepam comparative anxiety study

[28] FDA
Semax briefing document

[29] FDA
Emideltide DSIP briefing document

[30] DailyMed
SCENESSE afamelanotide implant

[31] Dorr and colleagues
Melanotan II phase 1 pilot study

[32] DailyMed
VYLEESI bremelanotide

[33] DailyMed
PREGNYL chorionic gonadotropin

[34] Grant and colleagues
NAD infusion metabolome pilot

[35] FDA
Ingredients suitable for sterile compounding

[36] Weschawalit and colleagues
Oral glutathione and skin properties

[37] FDA
Glutathione sterile injectable ingredient alert

[38] Billon and colleagues
Synthetic ERR agonist and exercise response

[39] CDC
Injection safety observation tool

[40] NXT LVL Research
GLOW 70 mg product page

[41] NXT LVL Research
TB 500 plus BPC 157 blend product page

[43] Eli Lilly and Company
TRIUMPH 2 detailed phase 3 retatrutide results